ST. JUDE NEUROLOGICAL INSTITUTE & ADVANCED BRAIN CLINIC

Department of Clinical Neuroscience • Parkinson's Disease & Deep Brain Stimulation (DBS) Candidacy Dossier

Patient Name:	Clara Oswald	**Patient Code / MRN:**	PT-9204
Date of Birth:	1958-03-14 (Age 68)	**Gender:**	Female
Primary Diagnosis:	Idiopathic Parkinson's Disease (Stage III)	**Attending Physician:**	Dr. Sarah Jenkins, MD
Evaluation Date:	2026-08-30	**Clinic:**	Movement Disorders Center

CLINICAL DISEASE TRAJECTORY & MEDICATION MOTOR FLUCTUATIONS

68-year-old female with 8-year history of idiopathic Parkinson's disease presenting with progressive motor fluctuations, 'wearing-off' phenomenon, and peak-dose dyskinesias limiting functional independence despite Carbidopa/Levodopa dose adjustments (taking 250/25 mg 5 times daily).

LEVODOPA CHALLENGE TEST & UPDRS SCORES

• MDS-UPDRS Part III (Motor Score OFF Medication): 44 points (severe resting tremor, rigidity, bradykinesia).
• MDS-UPDRS Part III (Motor Score ON Medication): 16 points (63.6% motor score improvement, demonstrating robust levodopa responsiveness, exceeding the 30% candidacy threshold for surgical DBS).
• Neuropsychological Evaluation: MoCA Score 28/30, indicating absence of dementia or significant frontal-executive impairment.

STEREOTACTIC TARGET ASSESSMENT

High-resolution 3T MRI volumetric brain protocol completed without motion artifact. Subthalamic Nucleus (STN) bilaterally identified as the primary therapeutic target for bilateral DBS electrode implantation.

MULTIDISCIPLINARY SURGICAL DECISION

Surgical board unanimously approves patient for bilateral STN Deep Brain Stimulation. Scheduled for staged frame-based bilateral lead placement next month.